

This essay was for a Global Seminar class I took in Edinburgh, Scotland on the History of Public Health. The prompt was to choose a historical site in the city and elaborate on the historical context and significance of the site in relation to the history of medicine and public health in Edinburgh. It encouraged me to explore the city and practice skills in historical research and analysis.

The Craiglockhart Poorhouse

Throughout history, socioeconomic status and health have been and continue to be inextricably intertwined. More specifically, poverty has led to illness, and illness has led to poverty. As the complexities of this dynamic are becoming increasingly better researched and understood, it is important to consider the legacies of past practices and perceptions. During the Victorian era, “epidemic disease was a sign of poor sanitary practices which were tied to poor economic and moral practices”, and this same dirt was seen as hindering one’s ability to “preserve independence from [one’s] surroundings”, therefore leading to “immorality” (Gilbert 13-14). As removing “filth” from an area was seen as a method to improve “the health of the social body”, “individual humans who live in slums were...drained of agency and massified” in order to present city conditions to sanitary establishments—that is, high densities of individuals living in poverty were categorized as a singular entities of filth, often presented as dark blemishes on maps (Gilbert 14). With this historical context of how Victorians thought of the link between poverty and disease, the history of Poor Laws in Scotland and the institutions that emerged as a result reflect a broader history of medicine and public health that is relevant on a global scale.

As middle and upper class Victorians grew resentful of the increasing cost of caring for the poor, governments intervened. The new Poor Law of 1834 was enacted as a

new system of poverty alleviation that applied to both England and Wales. This law grouped parishes into unions and required each union to build a workhouse for their local area if one did not already exist (“1834 Poor Law”). The conditions inside these workhouses were intentionally made harsh in order to deter impoverished individuals from “exploiting” government aid, making workhouses a last resort for survival (“1834 Poor Law”). Because Scotland’s judicial system remained separate from England after the Act of Union in 1707, the New Poor Law of 1834 did not apply to Scotland (“Caring for the Poor”). However, in 1845, Scotland enacted its own poor law. While the 1834 and 1845 poor laws were very similar, there existed key differences. Unlike the 1834 Poor Law in England and Wales, the poorhouses (the Scottish equivalent to workhouses) established as a result of the Scottish Poor Law Act did not require inhabitants—or inmates, as they were called—to cover the cost of their care through their labor; additionally, the Scottish Act *excluded* able-bodied individuals from applying for relief (“Caring for the Poor”). Prior to the 1845 Act, Scotland’s welfare system had not been centrally controlled. After 1845, the Act’s central Board of Supervision meant “there was always a Scottish institution able, and indeed with a statutory responsibility, to monitor the condition of the poor and ensure their needs were met” (Levitt xi). The 1867 Public Health Act included control of epidemic disease under the Board’s supervision, meaning “the Board had a duty to monitor sanitary conditions and ensure some modicum of public health” (Levitt xi). While the Board could not directly alter the way poorhouses were managed, the Board held the power to “determine whether any relief offered was adequate” and, upon threat of closure, had the ability to ensure adequacy of the bare minimum, such as “[a]dequate feeding, the separation of the sexes, the provision of

minimum sleeping accommodation, some modicum of medical care, and a general feeling of ‘a well regulated institution’” (Levitt xxii). Of the relief offered by poorhouses, the offering of medical care proved to be one of the most influential.

In Scotland, poorhouse relief quickly became synonymous with medical care. During the era of poorhouses hospitals were far from common, and private medical treatment was inaccessible for the poor (“Scottish Poorhouses”). However, this care was far from perfect—this can be seen through countless accounts from the time. One account from 1865 describes one of many examples, in which a five year old child died at the Old Monkland Poorhouse:

“To lock five children who were under medical treatment, into a probationary ward by themselves, at a distance from other inmates and with no nurse within call, from seven in the evening until the same hour in the morning, was a most objectionable arrangement...the post mortem examination showed that he must have been suffering for several days. Yet it is said that the nurse, a pauper inmate employed to attend to those children...never discovered that the boy was suffering from any other disease than the itch, though she put them to bed, after he had rejected his food, within a few hours of his death” (Levitt)

Another example of health in poorhouse conditions is an 1872 account from a Visiting Officer:

“it is not an exaggeration to say that almost every Poorhouse child in Scotland, whom I was able to inspect, was found affected with lice, and that in hardly a single Poorhouse was it deemed necessary either to fumigate or wash the clothing removed from the persons of inmates on admission” (Levitt)

Of course, there were efforts to solve health issues in poorhouses, as exemplified later in that same Visiting Officer’s 1872 account:

“There is now no Poorhouse in Scotland in which the old clothing is stored unwashed, and in nearly all Poorhouses it is also fumigated, while in these houses in which no fumigator exists, dirty clothing is either otherwise treated or burnt” (Levitt)

Despite the fact that the medical assistance provided through poorhouse relief was far from ideal, many had no other options at the time. In 1948, the Poor Law was abolished by the National Assistance Act and a financial safety net was established; while the medical functions of poorhouses were widely replaced by hospitals, many hospitals were established through the expansion of former poorhouses into hospitals (“Scottish Poorhouses”).

Of the poorhouses in Scotland, the Craiglockhart Poorhouse was one of the largest in Edinburgh. Replacing what was originally the Edinburgh City Poorhouse, the Craiglockhart Poorhouse opened in 1870 after the enactment of the new Poor Law. In a

1907 annual report by Mr. W. Bennet (the governor of Craiglockhart Poorhouse), “inmates” are described as being “employed as tailors, shoemakers, carpenters, painters, blacksmiths, plumbers, warders, scrubbers, at wood-splitting, flax-teasing, sewing and laundry work; and on the farm, garden, and roads” (qtd. in Higginbotham). Additionally, “[g]irls are trained in sewing, knitting, and housework”, while “[b]oys are trained in garden work during summer” (qtd. in Higginbotham). The Craiglockhart Poorhouse consisted of two buildings: the “main” poorhouse and an adjacent infirmary. Divided by sex, “[t]he male wing had divisions for ‘old men of good character’, ‘dissolute men’, ‘doubtful old men’, and ‘boys’, with similar divisions in the women’s quarters” (Higginbotham). The infirmary consisted of “four pavilions arranged in pairs for ordinary cases” and an additional detached pavilion “to be used exclusively as a fever hospital” (qtd. in Higginbotham). In 1901, “open air treatment” was proposed for handling tuberculosis in the Craiglockhart Poorhouse. The following is an extract from the Board’s reply to a 1901 inspection report:

“The ordinary career of a consumptive in the ranks of pauperism is to pass into and out of the Poorhouse, a superficial recovery probably following each period of residence, but on each occasion from a lower level, so that after time entire disablement, and finally death ensue...The object of Sanatorium treatment is so to strengthen the vitality of the tissues as to stop the inroads of the bacillus of tubercle, and, ultimately to destroy it. This is sought to be accomplished –

- (1) By placing the patient in physical circumstances which promote digestion and assimilations, viz: as much as possible in open air.
- (2) By the liberal supply of wholesome and appetising food.
- (3) By a general regimen (exercise, rest, etc) adjusted to the conditions of each patient.

Special structural arrangements, a special dietary, and careful supervision are therefore necessary...the trial of the open-air treatment proposed to be made at Craiglockhart is being instituted on the suggestion of Dr Carmichael, Consulting Physician” (qtd. in Higginbotham).

In a 1911 publication of the British Medical Journal, Sydney Holland writes a correspondence on “Poor Law Infirmaries And Hospitals” in which the following about the Craiglockhart Poorhouse is described:

“I got an order from Mr. John Burns and the Local Government Board at Edinburgh to visit the Craiglockhart Poorhouse, and I found a state of things there which could not be found in the worst managed voluntary hospital in the United Kingdom. I found 800 inmates, all certified by the medical officer to be in need of medical relief...of these 800, 280 were really ill and on the hospital side, there were 90 patients with phthisis. To 42 of such patients there was one nurse by day and one nurse by night, though they were in wards a considerable distance apart! In the other part of the hospital there was only one night nurse between four wards with eighteen beds in each!...there was but one resident medical officer, and

she was a young lady of 26, holding her first appointment...this young woman had under her sole charge the most filthy ward I have ever seen in my life, full of men with bad syphilis!”

It is also worth noting that the Edinburgh City Hospital for Infectious Diseases was built near the Craiglockhart Poorhouse. Though the Craiglockhart Poorhouse was by no means exemplary in the field of public health, it was—as were other poorhouses in the country—a source of medical care for the poor nonetheless.

The way in which poorhouses provided medical care for the poor—paired with the expansion of poorhouses into hospitals—characterizes the relevance of poorhouses in the broader history of medicine and public health in Scotland and the United Kingdom. This intertwined history is also a reflection of health issues at a global scale, as the field of Global Health tends to focus on the health of populations at lower socioeconomic statuses, and poverty alleviation is one approach to improving population health still in use today. While it is important to recognize the link between socioeconomic status and health, it is imperative that understandings remain nuanced. An individual suffering a correlation is not automatically the causation—collective healing is only possible through the recognition of collective burden and consequent collective action.

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